Water Care

Annual Project Results Report
Supported by Global Affairs Canada

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Taps for Rights: Water, Sanitation and Menstrual Health for Sexual and Reproductive Health and Rights

Annual Project Results Report, Year 3

Project number	SYN-P0417 (Purchase Order SYN-7417)
Countries	Republic of Nimbara (Tenkori and Diafo districts, Kessara Region); Sundara (Nadipur District, Rivana Province)
Implementer	Water Care, with Water Care Nimbara and Water Care Sundara
Reporting period	1 April 2020 to 31 March 2021
Date of report	27 May 2022
Project start and end	21 June 2019 to 31 March 2023 (activities); agreement expiry 30 September 2023
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1. Executive summary

Year 3 (1 April 2021 to 31 March 2022) was the year Taps for Rights moved from planning to visible change. In the third year of this four-year project, Water Care and its partners brought water into menstrual hygiene spaces in 17 more schools and 6 more health care facilities, trained 486 teachers and health workers, and saw menstrual health appear in national guidance for the first time in Nimbara.

Highlights from the Year 3 midline survey and monitoring data:

- 97% of national government and civil society stakeholders now have positive knowledge of menstrual health.
- 22 targeted schools (55%) now include gender-sensitive sanitation for menstrual health in their school plans, a 52% increase from baseline.
- 64% of targeted schools and health care facilities had water available on premises at the time of the monitoring visit, up from 38% at baseline.
- District budgets for water, sanitation and menstrual health in Tenkori rose from 4.4% to 7.9% of total district spending.
- 137 community champions are now active, exceeding the project target.
- Girls' school attendance during menstruation has improved significantly in targeted schools.

The year was not without challenges. Insecurity in parts of Kessara Region closed eight targeted schools in Nimbara, and monsoon flooding in August 2021 damaged facilities in four schools in Nadipur District, Sundara. Our Canadian youth engagement partner, Northern Youth Exchange, ceased operations in November 2021. Lessons from these challenges are set out in sections 8 to 10.

2. Changes to context

Nimbara

The security situation in the south of Kessara Region worsened during Year 3. Armed attacks in two municipalities of Diafo District displaced families into Tenkori District and led education authorities to close eight of the project's 25 targeted schools in Nimbara in September 2021. With district education officials, the team selected ten replacement schools in Tenkori and in neighbouring Bamiro District, which was not part of the original project area. Inflation of around 12% raised the cost of construction materials and transport. National elections are scheduled for November 2022.

Sundara

Heavy monsoon rains in August 2021 flooded large parts of Nadipur District. Four targeted schools were used as shelters by displaced families; latrines and two rainwater tanks were damaged by flood water and overuse. Schools in the district were closed for six weeks. On the positive side, the Rivana Province education department added a menstrual hygiene indicator to its Education Management Information System in January 2022, following advocacy by the project. Discussion of "rights" in sexual and reproductive health remains sensitive with provincial authorities, so the Sundara team frames its work as sexual and reproductive health and menstrual hygiene.

COVID-19

COVID-19 restrictions eased in both countries during Year 3, but large gatherings were still limited until October 2021, which delayed community events and trainings in the first half of the year.

3. Progress on outcomes

Values below come from the midline survey (February 2022, households, schools and health care facilities in all three districts) and from routine monitoring. Baselines and targets are from the approved PMF. See Annex 1 for the full table.

3.1 Ultimate outcome

1000: Improved comprehensive sexual and reproductive health and rights (SRHR), with a focus on women and adolescent girls, through water, sanitation and menstrual health interventions in Nimbara and Sundara.

Indicator	Baseline	Target	Year 3 (midline)
1000.1 % of women and adolescent girls in targeted communities reporting positive menstrual hygiene management practices	Girls <15: 35% Girls 15-19: 44% Women 20+: 56%	75%	Girls <15: 52% Girls 15-19: 60% Women 20+: 66%

All age groups improved. Women and girls most often cited water on site and a lockable door as the changes that made the biggest difference. Progress is slowest among girls under 15, many of whom have only recently started menstruating.

3.2 Intermediate outcomes

1100: Increased government and civil society responsiveness to the menstrual health needs of women and adolescent girls.

Indicator	Baseline	Target	Year 3
1100.1 # of actions by national, provincial or local government officials to address the menstrual health of women and adolescent girls as a result of project assistance	0	18	7
1100.2 # of national and sub-national policies, guidelines and strategies relating to, or including, menstrual health strengthened or enacted as a result of project assistance	0	3	2
1100.3 # of actions by civil society organizations to address the menstrual health of women and adolescent girls as a result of project assistance	0	6	21

The two policies counted under 1100.2 are Nimbara's national guideline on menstrual hygiene in schools, validated in November 2021, and the new menstrual hygiene indicator in the Rivana Province Education Management Information System in Sundara. Civil society action has far exceeded expectations: women's associations in Tenkori and youth groups in Nadipur organized 21 public actions, including Menstrual Hygiene Day marches and a radio debate with district officials.

1200: Increased utilization of gender-sensitive WASH facilities and menstrual hygiene product supplies in schools, health care centres and communities.

Indicator	Baseline	Target	Year 3
1200.1 % of adolescent girls in targeted schools reporting sanitation and hygiene facilities are acceptable for them	1%	80%	31%
1200.2 % of women and adolescent girls in targeted health care facilities reporting sanitation and hygiene facilities are acceptable for them	0.3%	70%	6%
1200.3 % of women and adolescent girls in targeted communities using acceptable hygienic products for menstruation when needed	Girls <15: 21% Girls 15-19: 28% Women 20+: 36%	0.50%	Girls <15: 61% Girls 15-19: 66% Women 20+: 70%
1200.4 % of targeted schools and health care facilities with water available on premises at the time of visit	38%	85%	64%

Acceptability of school facilities has risen sharply where water has been brought into the menstrual hygiene room. In health care facilities, progress is much slower: rehabilitation works in 14 of the 20 facilities are not yet complete, and women report that toilets are often locked or reserved for staff.

3.3 Immediate outcomes

Indicator	Baseline	Target	Year 3
1110.1 % of targeted national government and civil society stakeholders with positive knowledge of menstrual health (by sex)	Women: 70% Men: 68%	35%	Women: 96% Men: 90% Overall: 93%
1120.1 % of local government and civil society stakeholders with positive knowledge of menstrual hygiene management (by sex)	Women: 40% Men: 24%	70%	Women: 66% Men: 58%
1120.2 % of local government budgets in target areas allocated to menstrual health in schools and health care facilities	0%	15%	1.2%
1120.3 # and % of targeted schools and health care facilities with gender-sensitive sanitation for menstrual health included in plans	Schools: 1 (3%) HCFs: 1 (5%)	50%	Schools: 22 (55%) HCFs: 9 (45%)
1210.1 % of health centre staff and teachers with positive knowledge of menstrual hygiene management (by sex)	HCF staff M 55%, W 41% Teachers M 42%, W 28%	80%	HCF staff M 81%, W 74% Teachers M 66%, W 70%
1210.2 # and % of targeted schools with provision for menstrual hygiene management (water, private space, disposal)	6 (15%)	60%	24 (60%)
1210.3 # and % of targeted health care facilities with basic hygienic and sanitation facilities and menstrual hygiene products	1 (5%)	50%	11 (71%)

1220.1 # of local private sector actors producing menstrual hygiene products (by sex)	0	Not yet set	Women: 38 Men: 4
1230.1 % of community members with positive knowledge of menstrual hygiene management (by sex and age)	Girls 10-14: 14%, 15-19: 13%, women 24% Boys 10-14: 8%, 15-19: 14%, men 15%	70%	Girls 10-14: 41%, 15-19: 47%, women 52% Boys 10-14: 22%, 15-19: 35%, men 38%
1230.2 # of local actors promoting menstrual hygiene management in target communities as a result of project assistance (by sex)	0	150	Women: 96 Men: 41

Knowledge among national stakeholders is now very high. Local private sector production of menstrual products is growing: 42 entrepreneurs, almost all women, are making and selling reusable pads at nine sales points. A target for this indicator will be set once market data from Year 4 are available.

4. Theory of change, logic model and PMF

There were no changes to the theory of change or logic model in Year 3. To reflect the project's work with provincial authorities in Sundara, the wording of indicator 1100.2 now covers "national and sub-national" policies. Indicator 1200.4 on water availability was added to the PMF during the baseline and is reported for the second time this year.

5. Reach

By 31 March 2022, Taps for Rights had directly reached 19,240 people, 82% of the life-of-project target of 23,450 direct beneficiaries.

Direct reach to date	Nimbara	Sundara	Total
Girls 10-19	6,410	3,410	9,820
Women 20+	1,180	2,230	3,410
Boys 10-19	3,950	810	4,760
Men 20+	820	430	1,250
Total	**12,360**	**6,880**	**19,420**

Indirect reach (radio listeners and participants in commemoration days) is estimated at 41,600 people. It is not disaggregated by sex this year because radio audience data were not available by sex.

6. Outputs and activities

1100: Responsiveness of government and civil society

- **1111 Policy gaps identified:** gap analyses from Year 1 were updated in both countries to include water supply standards for schools.
- **1112 Ministries supported:** six ministries engaged (water, education, health and gender in Nimbara; education and health in Sundara). Technical support to Nimbara's Ministry of Education led to the national guideline on menstrual hygiene in schools.

- **1113 Women influencers:** 14 women influencers (parliamentarians, journalists, faith leaders) engaged; three spoke publicly on menstrual health during the year.
- **1121 Local authorities trained:** 212 district officials and service providers trained (88 women, 124 men).
- **1122 District gaps mapped:** menstrual health gap maps completed in Tenkori, Diafo and Nadipur and presented at district planning meetings. Tenkori District Council created a budget line for menstrual health in schools in its 2022 budget.
- **1123 WASH at Work:** campaigns run in four markets, with market committees agreeing to keep toilets open and supplied with water.

1200: Use of facilities and products

- **1211 Facilities rehabilitated and adapted:** water and sanitation facilities rehabilitated in 17 schools and 6 health care facilities in Year 3 (29 schools and 11 health care facilities to date). Works included 9 rooftop rainwater harvesting systems, 3 solar-powered pumps on existing boreholes, and 22 menstrual hygiene rooms with a tap inside. At five replacement schools in Nimbara that had no usable toilets, the project constructed new four-stance latrine blocks with a menstrual hygiene room. A Construction Validation Plan for the replacement schools will be submitted to Global Affairs Canada in the second quarter of Year 4.
- **1212 Staff trained:** 486 teachers and health workers trained (251 women, 235 men).
- **1213 Students sensitized:** 6,940 students reached through peer-to-peer sessions (4,310 girls, 2,630 boys).
- **1221 Supply chains mapped:** completed in Year 2; updated in Sundara after the floods.
- **1222 Entrepreneurs supported:** 42 entrepreneurs (38 women, 4 men) trained to make and sell reusable pads; nine sales points opened.
- **1231 Awareness campaigns:** radio spots on three stations; 26 community events.
- **1232 Community champions:** 137 champions identified and trained (96 women, 41 men).

7. Gender equality, environment and human rights

Gender equality. Water Care is committed to gender equality. Global Affairs Canada has committed to ensure that 85 percent of its bilateral international assistance is implemented through initiatives that advance gender equality, and Taps for Rights contributes to this target. Women make up 52% of members of the 40 school water and sanitation committees supported by the project. In Sundara, separate sessions for men and boys were introduced after fathers asked to learn more.

Environment. Small incinerators for used pads were installed in six health care facilities; ash is buried on site. Flood-resilient designs (raised plinths, sealed pit linings) were used in Nadipur District after the August 2021 floods. The Environmental Management Plan has not been updated since the PIP.

Human rights. The project promotes the human rights to water and sanitation. Facilities are designed with ramps and handrails for people with disabilities; 18 of the 22 new menstrual hygiene rooms are wheelchair accessible.

8. Risk management

Risk	Rating	Response in Year 3
Insecurity in Kessara Region (Nimbara)	High (was Medium)	Eight schools closed; ten replacement schools selected in Tenkori and Bamiro districts with district education officials. Staff travel restricted to daylight hours with security clearance.

Flooding in Nadipur District (Sundara)	High	Damaged facilities in four schools repaired; flood-resilient designs adopted for all remaining works.
Cultural and religious resistance to discussing menstruation	Medium	Early engagement of religious and traditional leaders; separate sessions for men and boys.
Loss of youth engagement partner	Medium	See section 10.
Rising prices of materials and transport	Medium	Bulk procurement; some activities rescheduled.

9. Lessons learned

- Water on site matters more than any other single feature of a menstrual hygiene room. Rooms without a tap are used far less.
- Health care facilities need a different approach from schools: toilets are often reserved for staff, so management agreements are needed before works start.
- Community champions are often students or unemployed young people. Many pay their own transport to project activities, which is not sustainable.

10. Project management and governance

Water Care is the sole signatory and leads overall management, reporting and compliance. Water Care Nimbara and Water Care Sundara implement the project with local partners AFST and RDSN. Bloom Health Collective provides technical support on menstrual health training.

Our Canadian youth engagement partner, Northern Youth Exchange, ceased operations in November 2021. To keep youth engagement on track, its project lead was retained as an individual consultant from December 2021 under the same terms of reference. The Global Affairs Canada project officer was informed of this change by telephone in January 2022.

The Project Steering Committee met twice in Year 3. In October 2021 it approved the replacement schools, including those in Bamiro District, and the reallocation of funds described in section 11.

11. Financial summary

Spending is shown as a percentage of the Year 3 budget for each line and, in the last column, as cumulative spending against the life-of-project budget. Figures are in Canadian dollars equivalent; costs incurred in Nimbaran francs and Sundaran rupees were converted at the month-end bank rate.

Budget line (Appendix C)	Year 3 spent as % of Year 3 budget	Cumulative spent as % of life-of-project budget
1.1 Remuneration: Organization's employees	94%	63%
1.2 Remuneration: Local employees	101%	66%
1.3 Fees: Subcontractors	72%	51%
1.5.1 Travel costs	38%	29%

1.5.4 Other training costs	61%	48%
1.5.5 Goods, assets and supplies	168%	97%
1.5.6 Project administration costs	104%	70%
1.5.7 Other direct costs	88%	55%
1.5.8 a) Construction: structural works	132%	81%
1.5.8 b) Construction: non-structural works	88%	64%
1.6 Overhead (12%)	92%	60%
Total	**89%**	**61%**

Variances. Goods, assets and supplies were higher than planned because of hygiene kits and pad-making materials for flood-affected schools in Sundara and replacement schools in Nimbara. Funds were reallocated from Other training costs and Travel costs, which were underspent because of COVID-19 restrictions on gatherings. Structural construction was higher than planned because of the new latrine blocks at replacement schools and flood repairs. A request for Global Affairs Canada's approval of the reallocation was sent on 3 June 2022 and is pending.

Water Care contribution. Water Care's cash contribution to date is 8.7% of eligible direct costs incurred. We expect to reach the required contribution by the end of the project.

Advances and interest. On 30 March 2022, Water Care requested the advance for April to September 2022. The periodic financial report for April to September 2021 was submitted on 4 April 2022, and the report for October 2021 to March 2022 on 13 May 2022. Interest earned on advances during Year 3 was used to buy additional hygiene kits for flood-affected schools.

12. Upcoming reports and next steps

Report or action	Due date	Status
Annual Work Plan, Year 4	15 July 2022	In preparation; date agreed with the project officer by telephone
Audited financial statements, fiscal year 2021-22	30 September 2022	Audit under way
Construction Validation Plan, replacement schools	Q2 of Year 4	In preparation
Project Operations Report and Periodic Financial Report, April to September 2022	14 November 2022	Not started
Disposal of Assets Plan	28 February 2023	Not started
Final Results and Operations Report and Final Financial Report	15 May 2023 (45 days after end of activities)	Not started

Next steps. Because of delays caused by insecurity, flooding and COVID-19, rehabilitation works in 14 health care facilities and community activities in the replacement schools cannot be completed by 31 March 2023. Water Care intends to request a twelve-month no-cost extension of project activities to 31 March 2024 and will discuss this with the project officer in Year 4.

Annex 1. Updated Performance Measurement Framework (summary)

Code	Indicator (short)	Baseline	Target	Year 3	Data source
1000.1	% women and girls with positive MHM practices	35% / 44% / 56%	75%	52% / 60% / 66%	Household survey
1100.1	# government actions on menstrual health	0	18	7	Advocacy tracker
1100.2	# national and sub-national policies including menstrual health	0	3	2	Policy documents
1100.3	# civil society actions on menstrual health	0	6	21	Advocacy tracker
1200.1	% girls finding school facilities acceptable	1%	80%	31%	School survey
1200.2	% women and girls finding HCF facilities acceptable	0.3%	70%	6%	Exit interviews
1200.3	% using acceptable hygienic products	21% / 28% / 36%	0.50%	61% / 66% / 70%	Household survey
1200.4	% schools and HCFs with water on premises	38%	85%	64%	Facility checklist
1110.1	% national stakeholders with positive knowledge	W 70% / M 68%	35%	W 96% / M 90%	KAP survey
1120.1	% local stakeholders with positive knowledge	W 40% / M 24%	70%	W 66% / M 58%	KAP survey
1120.2	% local budgets for menstrual health	0%	15%	1.2%	District budgets
1120.3	# and % schools and HCFs with MH in plans	1 (3%) / 1 (5%)	50%	22 (55%) / 9 (45%)	Facility plans
1210.1	% staff and teachers with positive knowledge	See section 3.3	80%	See section 3.3	KAP survey
1210.2	# and % schools with MHM provision	6 (15%)	60%	24 (60%)	Facility checklist
1210.3	# and % HCFs with basic facilities and products	1 (5%)	50%	11 (71%)	Facility checklist
1220.1	# private sector actors producing MH products	0	Not yet set	W 38 / M 4	Partner records
1230.1	% community members with positive knowledge	See section 3.3	70%	See section 3.3	Household survey
1230.2	# local actors promoting MHM	0	150	W 96 / M 41	Partner records